

--

Name: ساميه على ابراهيم عبدالله National ID: 27302250101021	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
←



Patient Name : الأستاذ / سامية على ابراهيم عبدالله	Request No. : 13000435
Patient No. : 27302250101021	Request Date : 28/04/2022
Path No. : SR-315/22	Verification Date : 07/05/2022
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Breast cancer and Uterine cancer

Nature of the specimen: Slides Revision

Gross Examination:

11 Blocks (3769) + 14 Slides

Microscopic examination:

* Mastectomy Slides:

- Histological type: Invasive Ductal Carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1)
- Angioinvasion: Present
- o Skin involvement: Tumour cells reaching base of nipple with free epidermis
- Additional Pathological Findings: Positive perineural invasion
- Biomarkers

Examination of ER stained slide show intense nuclear staining in 70% of the tumor cells.

Examination of PR stained slide show intense nuclear staining in 90% of the tumor cells.

Examination of HER2 stained slide show intense staining in >10% of tumor cells with complete membranous staining.

Examination of Ki67 stained slide show nuclear staining in 45% of tumor cells

* Total Hystrectomy with unilateral salpingectomy Slides:

Microscopic examination showed Endometrium with Endometroid carcinoma (villoglandular type), infiltrating more than half of the myometrium, with vascular emboli. Free cervix, with chronic non-specific cervicitis and Nabothian cyst. Free tube with salpingitis and pyosalpinx.

Diagnosis:

Breast cancer and Uterine cancer, Slides Revision:

Verified by :

dr.menna atef

قسم الباثولوجي



Patient Name : الأستاذ / سامية علي ابراهيم عبدالله	Request No. : 13000435
Patient No. : 27302250101021	Request Date : 28/04/2022
Path No. : SR-315/22	Verification Date : 07/05/2022
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

- * Mastectomy Slides:
 - Invasive Ductal Carcinoma GII
 - Tumour cells reaching base of nipple with free epidermis
 - ER: Positive (score 8/8).
 - PR: Positive (score 8/8).
 - HER-2: Positive (score 3+)
 - Ki67: Positive in 45% of tumor cells.
- * Total Hystrectomy with unilateral salpingectomy Slides:
 - Endometroid carcinoma (villoglandular type), infiltrating more than half of the myometrium.
 - Free cervix, with chronic non-specific cervicitis and Nabothian cyst.
 - Free tube with salpingitis and pyosalpinx.

Prof. Lubna Omar

Dr. Menna Atef

Verified by :
dr.menna atef

Printing Date : 17/05/2022 08:47:59

Page 2 of 2

Printing By : Ali mostafa

ا.د. أحمد كمال الكسري
ا.د. محمد أبو الهدى درويش
ا.د. عمر حسين عمر
ا.د. وديد حسين طنطاوى
ا.د. ياسر عبد العظيم عباس
ا.د. أحمد محمد منيب
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

MRS.: SAMIA ALI IBRAHIM

ID: 1835476

DEAR SIR

DATE: 14/04/2022

¹⁸F-FDG PET/CT EXAMINATION

History:-

49-years old female patient, presented with history of right breast cancer (invasive ductal carcinoma grade-III), as well as endometrial cancer (endometrioid adenocarcinoma grade-III), for which right mastectomy and hysterectomy were done three weeks ago and referred for PET/CT assessment.

Procedure:-

- 9.5 mCi ¹⁸F-FDG was administered to the patient and imaged after appropriate time.
- The blood glucose level was 150 mg/dl.
- A low dose non-contrast CT for attenuation correction and anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- A diagnostic 128 MDCT non contrast examination was taken of the same regions. Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.

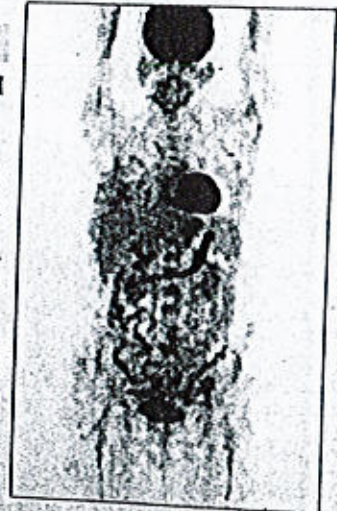
PET/CT FINDINGS:-

A) Head and Neck:-

- The neck is devoid of any abnormally hypermetabolic lymph nodes.
- The visualized portions of the brain exhibited normal FDG biodistribution.
- Physiological FDG uptake is seen in the oropharynx, salivary glands and larynx.
- No parenchymal thyroid focal lesions could be elicited.
- Mild bilateral maxillary sinusitis is noted.

B) Chest:-

- The right mastectomy operative bed shows mildly active subcutaneous fat stranding with small localized fluid collection (seroma), measuring about 10x22x34mm [reactive post operative changes], yet, no hypermetabolic soft tissue lesions detected that would account for residual neoplasia.



أ.د. أحمد كمالي
أ.د. محمد أبو الهدى
أ.د. عمر حسين
أ.د. وحييد حسين طنط
أ.د. ياسر عبد العظيم عباس
أ.د. أحمد محمد منيب
أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس

- No hypermetabolic left breast soft tissue lesions detected.
- Mildly active left axillary lymph node is noted, measuring about 1.4cm and achieving up to 3.7 SUV max.
- No other metabolically active lymph nodes detected.
- No pulmonary nodules or pleural effusion seen.
- Inactive fibrotic bands at right lung upper and middle lobes are noted (likely post radiation sequelae).
- Normal tracer biodistribution uptake is noted in the myocardium.



C) Abdomen & Pelvis:-

- The pelvic operative bed is clear and devoid of any hypermetabolic soft tissue lesions that would account for residual neoplasia.
- Mildly active fat stranding is seen along the lower anterior abdominal wall surgical incision line is noted (reactive post operative inflammatory changes).
- There is normal distribution of the radiotracer within the gastrointestinal and genitourinary systems without focal areas of abnormal metabolism.
- Physiological uptake in liver, adrenal glands, spleen and pancreas.
- Physiological excretion of FDG in the kidneys.
- No ascites or hypermetabolic lymphadenopathy noted.



D) Musculoskeletal:-

- No metabolically active osseous lesions could be detected along the surveyed skeleton.

د. أحمد كمال
د. محمد أبو الهدى
د. عمر حسين
د. وحيد حسين
د. ياسر عبد العظيم عباس
د. أحمد محمد منيب

أساتذة الأشعة التشخيصية
كلية الطب - جامعة عين شمس



CONCLUSION:-

- No evidence of hypermetabolic residual neoplasia at right mastectomy and pelvic operative beds, with expected mildly active postoperative changes, as foredescribed.
- Mildly active left axillary lymph node, that could be reactive or metastatic, for follow up.
- No otherwise active lesions detected elsewhere in the body.

MUCH OBLIGED

A PROF MOSTAFA ABDELKAWY

PROF DR. OMAR HUSSEIN

A. PROF. DR. SUSAN ADIL



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
طبية
17/5/2022

MDT FORM

17/5/2022

Name	سليم علي ابراهيم	MRN	00011711
Age		ID	002.000.0.001
Menopausal status		Phone	0109277670
OCP		marital status	
Co morbidity	DM on oral Dsmission	Family history	

Clinical Data

Bx:	R	L
ER: 8/8	PR: 8/8	HER2: +3
Ki67: 5-60%		
Sonomammography:		
Mets Workup:		
T N M		

Recommendations:

3 yrs ago Rt breast mass → RTBCS → PORT → Tamoxifen 5 yrs.
+ALND
2022 → Rt breast mass → Completion mastectomy. IDC GII PT3
→ Endometrial Thickness → Total hysterectomy & Salpingo-ooph.
→ endometrial adenocarcinoma GII infiltrating 1/2 of myometrium
FIGO IB, Free Cx & Free salpingo-ooph.

MDT OUTCOME & PLAN

4 AC 12 Tamoxifen/Hormonal
PORT for breast and VPI for
adjuvant (after chemo)

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
17/5/2022		17/5/2022	

Amr sh

17/5/2022

Patient's Name: Samia Ali Ibrahim Abdallah
Date: 2026-08-23
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Left Digital soft tissue mammography with high resolution sonography revealed:

Clinical summary: Status post right MRM & ALAND, for two years, routine follow up.

Findings:

Left breast.

- Moderate dense breast tissue . (ACR C)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/hipple/areola complex.
- One indeterminate nature left axillary LN that measures (20 x 18 mm) with thick cortex of 6 mm .

Right mastectomy bed.

- Clear mastectomy bed with no masses or fluid collections..
- Clear right axilla. No pathological supra or infra-clavicular lymph nodes.

- **Conclusion:-**
- Left breast (BIRADS 3): One indeterminate nature left axillary LN for follow up.
- Clear right mastectomy bed and axilla.

Recommendation:-

- For annual follow up.
- Prof .MONA ZAKY